

Comprehensive Clinical Records with Sensitive Information

CONFIDENTIAL PATIENT RECORD

Patient Name: \*\*\*\*\*

\*\*\*\* of Birth: \*\*/\*\*/\*\*\*\*

NHS Number: \*\*\* \*\* \*

\*\*\*\*\*: \*\* \*\* \*\* \*

Hospital Number: HSP-\*\*\*\*\*

Address: \*\*\* \*\*\*, \*\*\*\* \*

\*\*\*\*\*: \*\*\*\*\*

Email: \*\*\*\*\*@\*\*\*\*\*.\*\*. \*\*

Occupation: Mechanical \*\*\*\*\*

Employer: West Yorkshire Industrial Systems Ltd.

Emergency Contact: \*\*\*\*\* - Wife - \*\*\*\*\*

Past Medical History:

- Type 2 Diabetes Mellitus
- Chronic Kidney Disease Stage 2
- Hypertension
- Obstructive Sleep Apnoea
- Hypercholesterolaemia

Current Medications:

- Metformin 1000mg twice daily
- Insulin Glargine 28 units nightly
- Ramipril 10mg daily
- Atorvastatin 40mg nightly
- Aspirin 75mg daily
- Omeprazole 20mg daily
- Sertraline 50mg daily
- Salbutamol inhaler PRN

Allergies:

Penicillin (anaphylaxis), Codeine (rash)

Clinical Note:

Patient attended diabetic review clinic reporting worsening peripheral neuropathy and intermittent blurred vision. Recent

HbA1c measured at 10.4%. Blood pressure 168/98 mmHg. Weight 108kg, BMI 34.2.

Investigations:

Creatinine: 118 umol/L

eGFR: 62 mL/min

Urine ACR: Elevated

ECG: Sinus rhythm with left ventricular hypertrophy.

Plan:

Increase insulin dose gradually to 34 units nightly. Refer to retinal screening service and ophthalmology clinic. Arrange

diabetic foot assessment within 2 weeks.

Consultant: Dr. \*\*\*\*\*, \*\* \*

MENTAL HEALTH ASSESSMENT \*\*\*\*\*

Patient: \*\*\*\*\*

Date of Birth: \*\*/\*\*/\*\*\*\*

NHS Number: \*\*\* \*\* \*

Address: Flat 6, \*\*\*\*\* Apartments, \*\*\*\* \*

\*\*\*\*\*: \*\*\*\*\*

Email: \*\*\*\*\*@\*\*\*\*\*. \*\*

Marital Status: Divorced

Next of kin: \*\*\*\*\* (Mother) - \*\*\*\*\*

Presenting Complaint:

Patient referred following episodes of severe anxiety, panic attacks, insomnia, and depressive symptoms lasting

approximately 8 months.

Psychiatric History:

Previous diagnosis of Generalised Anxiety Disorder and recurrent depressive disorder. One previous psychiatric

admission in 2021 following overdose attempt.

Current Medications:

- Venlafaxine XR 225mg daily
- Diazepam 5mg PRN up to TDS
- Zopiclone 7.5mg nightly
- Propranolol 40mg twice daily

Social History:

Lives alone. Reports significant alcohol intake on weekends and intermittent cannabis use. Currently signed off work

from role as HR manager.

Risk Assessment:

Passive suicidal ideation reported without active plan. No psychotic symptoms identified.

Management Plan:

Urgent referral to community mental health team. Weekly psychological therapy sessions recommended.

Safety

planning discussed extensively.

Prepared By: Dr. \*\*\*\*\* , Consultant Psychiatrist

Medication Administration & Sensitive Data Sheet

Medication

Dose

Frequency

Prescriber

Morphine Sulfate

10mg

Every \* \*\*\* \*\*

\*\* . \*\*\*\*\*

Warfarin

5mg

Once daily

Dr. \*\*\*\*\*

Levothyroxine

75mcg

Morning

Dr. \*\*\*\*\*

Pregabalin

150mg

Twice daily

Dr. \*\*\*\*\*

Clonazepam

1mg

Nightly

Dr. \*\*\*\*\*

Oxycodone

20mg

Twice daily

Dr. \*\*\*\*\*

Insurance & Billing Information

Private Insurance Provider: \*\*\*\*\*

Policy Number: \*\*\*\*\*

Claim Reference: \*\*\*\*\*

Bank Sort Code: \*\*-\*\*-\*\*

Partial Account Number: \*\*\*\*\*

Outstanding Balance: Â£\*,\*\*\*.\*\*

Additional Confidential Notes:

Patient disclosed previous opioid dependency treated in 2019. History of missed follow-up appointments and medication

misuse concerns documented.

Authorised Clinician Signature: Dr. \*\*\*\*\*